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PRE-OPERATIVE ASSESSMENT & PREPARATION

Hypertension Before Elective Surgery

YG CG 311 FEB 22

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Key Points

- Hypertension should be optimised by the GP.
- If BP >180/110, there is a small increase in risk for adverse cardiac outcome
- Patients should **not** be cancelled if their BP is high on the day of surgery, provided the pre-operative readings have been satisfactory.

Definitions

	Systolic	Diastolic
Stage 1 Hypertension	140-159	90-99
Stage 2	160-179	100-109
Stage 3	180-209	110-119
Stage 4	>209	>119

POAC Management

- All patients with hypertension should have an ECG, U&Es and random glucose or HBA1C sent.
- The GP is best placed to optimise hypertension.
- If a GP recorded BP reading is available **<160/100** in the last 6months, there is no need to take into account any blood pressure measurements performed in pre-assessment. The GP's recorded BP should be documented instead.
- If not, record the patient's blood pressure in POAC. Measure blood pressure up to three times and record the lowest reading only. If **<180/110**, then allow to proceed to surgery.
- The lowest recording in clinic or from GP records should be used for decision making.

POAC BP Reading	Management
Stage 1 & 2	Do not postpone surgery. If no previous diagnosis, refer back to GP for assessment and management with a letter and a copy of their ECG and blood tests If no previous diagnosis.
Stage 3	Check compliance with medication Cancel surgery Refer back to their GP for assessment and management, with a letter and a copy of their ECG and blood tests.
Stage 4	Check compliance with medication Cancel surgery Arrange same day GP review, and inform POAC anaesthetist.

- Any new antihypertensive treatment should be embedded for 4 weeks, if possible.

BP Management on the Day of Surgery

The following refers to ward BP readings:

- If BP <180/110, proceed with surgery
- Stage 3 Hypertension: D/W 2nd on call Consultant before cancelling. Consider antihypertensive or anxiolytic medication, if appropriate.
- If BP >209/119, cancel surgery. Request ECG. If asymptomatic, arrange urgent GP follow up. If symptomatic, refer to on call physicians.

Urgent Surgery

- Obtain advice from POAC Anaesthetist

References

- AAGBI: The measurement of adult blood pressure and management of hypertension before elective surgery, March 2016